[redacted]

IN / OUT / PR PATIENT ADMISSION RECORD

[redacted]

ADM TIME:1537 [redacted] CONF: VIP:

PT. TYPE:ADM IN ADMIT PRI/SRC:EM / PR [redacted] FC:09

PATIENT INFORMATION

[redacted]

AGE: [redacted] RACE: WHITE

SEX: F [redacted] STATUS: D

REL: CATHOLIC LANG: ENGLISH

SPOUSE / NOK / COMPANION

[redacted]

PERSON TO NOTIFY

[redacted]

PATIENT EMPLOYER

DISABLED

UNK

UNK,CA XXXXX

[redacted] OCC:

GUARANTOR

[redacted]

RELTN: SELF

GUARANTOR EMPLOYER

DISABLED

UNK

UNK,CA XXXXX

[redacted]

OCCURRENCE CODES

[redacted]

CONDITION CODES

INSURANCE INFORMATION

[redacted]

SECONDARY:

TERTIARY:

[redacted]

POLICY #:

COVERAGE #:

INS PHONE #:

GRP#:

AUTH #:

AUTH DT: VER DT:

SUB:

RELAT: DOB:

POLICY #:

COVERAGE #:

INS PHONE [redacted]

GRP#:

AUTH #:

AUTH DT: VER DT:

SUB:

[redacted]

COVERAGE #:

[redacted]

AUTH #:NR/V

[redacted]

PHYSICIAN INFORMATION / DOCUMENTATION

[redacted]

Does Not Know

SELF REFERRED

REASON FOR VISIT/CHIEF COMPL:SOB

PRINCIPAL DIAGNOSIS:

PRINCIPAL OPERATION/PROCEDURE:

CONSULTATIONS:

FOR MR USE ONLY

Assembly [ ] ___

Analysis [ ] ___

Coding [ ] ___

Printed [ ] ___

Final Check [ ] ___

[redacted]

DISCH DATE [redacted]

ADVANCE DIRECTIVE:

[redacted]

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to [redacted] or FAX to [redacted] For referral questions, please contact an Illumination Referral Coordinator at [redacted]

Referring Party

Referring Hospital [redacted]

Dept/Floor [redacted]

Phone/Pager # [redacted]

Email: _____

Nursing Station [redacted]

Authorized by: _____

Phone #: _____

Patient Information

Patient Nam [redacted] MRN [redacted] Birth (dd/mm/yyy) [redacted]

Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: _____

Last 4 # of Social Security [redacted] Insurance: IEHP Preferred Language: Spanish

Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:

SOB

Admit [redacted] Any Wounds? ☐ Yes ☒ No

If yes, explain the number, location, size and stage:

Independent with Wound Care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No

Was PPD/TB Test or Chest X-Ray Performed? ☐ Yes ☒ No If yes, Date: _____ Results: not indicated

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	☒ Yes* ☐ No	Active Infection	☐ Yes* ☒ No
*Explain	2L - Continuous	*Explain	
Self Administer All Meds	☒ Yes ☐ No*	Ambulatory	☒ Yes ☐ No
Continent of Bowel & Bladder	☐ Yes ☒ No	Assistive Device	☐ Yes* ☐ No
Colostomy/Ileostomy	☐ Yes ☒ No	*Explain	
Foley Catheter	☐ Yes ☒ No	Hospice	☐ Yes* ☒ No
Diabetes	☐ Yes ☒ No	*Explain	
Insulin	☐ Yes ☒ No	Car	☐ Yes ☐ No
Diabetes related Oral Meds	☐ Yes ☒ No	Service Animal	☐ Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☒ No ☐ Yes

☐ Mental Health DX If checked, what type? _____

☐ Non-compliant ☐ Forgetful

☐ Cognitive Impairment ☐ Registered Sex Offender

☐ Other Explain _____

Length of Stay Authorized in Recupertive Care: _____ Days

ILLUMINATION STAFF ONLY

Date Received: _____ Reviewed by: _____

Approved: ☐ Yes* ☐ No** *Date: _____ *Authorization #: _____ **Reason for Denial: _____

Admission Date: _____ Admission Time: _____ Exit Date: _____

Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: _____ by _____ Total LOS ____

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Homeless Status Certification

This Homeless Status Certification Form will not be considered complete unless signed and dated by both the homeless services provider and the person making the request for Community Supports Services benefits.

SECTION 1: MEMBER

To be completed by the homeless Member making the request for Community Supports Services benefits

I [redacted] (full legal printed name) certify that I am an IEHP Member and meet HUDs definition of homelessness or at risk of homelessness as defined in 24 CFR [redacted]

By signing below, I acknowledge that I have read, understand, and agree with the statements below.

[redacted]y)

SECTION 2: COMMUNITY SUPPORTS SERVICE PROVIDER

To be completed by a qualified Community Supports Service Provider as defined

[redacted] Title: MSW [redacted]

Address of CS Provider: [redacted]

[redacted] State: CA [redacted]

I [redacted] _____ (Full legal printed name of CS Provider re[redacted]

_____ (Full legal name of IEHP Member) is a homeless person or homeless child or youth as defined in HUD definition of homelessness or at risk of homelessness as defined in 24 CFR [redacted] This Member resides:

- [x] In places not meant for human habilitation, such as cars, parks, sidewalks, abandoned buildings
- [ ] In an emergency shelter.
  - Name of Shelter: _____
  - Address: _____
- [ ] In transitional or supportive housing for homeless persons who originally came from the streets or emergency shelters.
- [x] In any of the above places, but is currently in a hospital or other inpatient institution. (Documentation of hospital or inpatient institution required)
- [ ] Is being evicted within a week from a private dwelling unit and no subsequent residence has been identified and the person lacks the resources and support networks needed to obtain housing (Documentation of eviction needed)
- [ ] Is being discharged within a week from an institution, such as a mental health or substance abuse treatment facility or a jail/prison, in which the person has been a resident for more than 30-consecutive days and no subsequent residence has been identified and the person lacks the resources and support networks needed to obtain housing. (Documentation from institution required)

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☐ Is fleeing a domestic violence housing situation and no subsequent residence has been identified and the person lacks resources and support networks needed to obtain housing.	
Today's Date (mm/dd/yyyy) [redacted]	[redacted] [redacted]

IEHP | Community Supports

Recuperative Care and Short-Term Post-Hospitalization Referral Form

Recuperative Care is a short-term residential care for Members who no longer require hospitalization but still need to heal from an injury or illness and who's condition would be exacerbated by an unstable living environment. This form is intended to be used for IEHP Members only. Recuperative Care and Short-Term Post Hospitalization Housing

Request Type

☒ Initial Request ☐ Extension ☐ Bed Transfer ☐ Resumption of Care (Returning from a hospitalization)

Is the Member Interested in participating in Community Supports?	**Placement Requested:**
☒ Yes ☐ No	☒ Recuperative Care ☐ Short-Term Post-Hospitalization Housing

Referring Contact Name/Title:	**Main Phone/Mobile #:**	**Email Address:**
[redacted]	[redacted]	

Member's First Name:	**Last Name:**	**M**	**IEHP ID:**
[redacted]		[redacted]	

Diagnosis (required):	**ICD Code:**
SoB	

Community Supports Services Eligibility Criteria ***(check all that apply)***

Recuperative Care- must meet both of the following:

☒ Individuals requiring recovery in order to heal from an injury or illness.
Must be determined by a provider to have medical needs significant enough to result in ED visits, hospital admission or other institutional care
AND
☒ Meet HUD Definition of Homelessness or at risk of homelessness.
Signed Homeless attestation by Member must be received as part of documentation required

Short-Term Post- Hospitalization Housing- must meet ALL the following:

☐ Members exiting an institution, recuperative care facility, inpatient hospitals (either acute or psychiatric or Chemical Dependency and recovery hospital), residential substance use disorder or mental health treatment facility, correctional facility, or nursing facility
☐ Meet HUD definition of Homelessness or at risk of homelessness
☐ Meet one of the following criteria:
- ☐ Are receiving Enhanced Care Management
- ☐ Have one or more serious chronic condition
- ☐ Have serious mental illness
- ☐ Are at risk of institutionalization or requiring residential services as a result of a substance use disorder

☐ Have ongoing physical or behavioral health needs as determined by a qualified health processional that would otherwise require continued institutional care if not for receipt of Short-Term Post- Hospitalization Housing

Recuperative Care and Short-Term Post-Hospitalization Housing combined cannot exceed a duration of six months per rolling 12-months.

Please note, if box is NOT checked, STOP. Member does not meet eligibility criteria. Please submit supporting documentation with the referral form.

Current Housing Status	**Current Region**	**Placement Location**
☐ Prioritized for permanent supportive Housing unit/rental subsid resource through the local homeless Coordinated Entr System or similar system ☒ Meets HUD definition of "homelessness" ☐ At risk of experiencing homelessness	☐ San Bernardino Proper ☐ West San Bernardino ☐ High Desert ☐ Low Desert ☐ Corona/Temecula/Hemet [redacted] ☐ Other: _____	Willing to be housed in another County: ☐ Yes ☒ No Willing to be housed in anywhere in current County: ☒ Yes ☐ No Willing to be housed in current city only: ☒ Yes ☐ No Previously in another housing program: ☐ Yes ☒ No Area(s) Member CANNOT live in:

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Serious Chronic Condition (check all that apply):	**Serious Mental Illness** *(check all that apply):*	**Substance Use Disorder** *(check all that apply):*
☐ Diabetes requiring insulin ☒ Uncontrolled Hypertension ☐ Congestive Heart Failure ☐ End-Stage Renal Disease ☐ Cancer requiring radiation or chemotherapy ☐ Other:	☐ Bipolar Disorder ☐ Schizophrenia ☐ Major Depressive Disorder ☐ Post-Traumatic Stress Disorder (PTSD) ☐ Obsessive Compulsive Disorder (OCD) ☐ Borderline Personality Disorder ☐ Other:	☐ Alcohol ☐ Opioid Abuse ☐ Heroine ☐ Cocaine ☐ Methamphetamines ☐ Other:
NOTE: Please include supporting documentation outlining how Member is unstable and requires recuperative care.	***NOTE: Please include supporting documentation outlining active signs and/or symptoms Member is displaying.***	***NOTE: Please include positive drug screen if available.***

Health Information

Does Member have impaired cognition? ☐ Yes ☒ No

Is Member able to ambulate at least 250ft with or without assistance?
☒ Yes ☐ No

Does Member use a wheelchair, walker, cane, etc. and if so, is Member independent?
☐ Yes ☐ No

Is Member independent with transfers?	☒ Yes	☐ No
Can Member self-administer all medications independently?	☒ Yes	☐ No
Is Member continent of both bowel and bladder?	☒ Yes	☐ No
Does Member take any medications requiring refrigeration?	☐ Yes	☐ No
Does Member require oxygen?	☐ Yes	☐ No
Does Member require ostomy care and or catheter care?	☐ Yes	☒ No
Does Member require a wound vac, Bipap, and/or Cpap?	☐ Yes	☒ No
Does Member require wound care?	☐ Yes	☒ No
Can Member perform wound care independently?	☐ Yes	☒ No
Does Member require IV antibiotics?	☐ Yes	☒ No
Can Member administer IV antibiotics independently?	☐ Yes	☐ No

NOTE: Please provide supporting documentation. If Member cannot perform wound care or administer IV antibiotics independently, Home Health arrangements must be arranged prior to admission to recuperative care.

Animals/Pets

Is Member requesting to bring an animal? ☐ Yes ☒ No

Is animal a service animal or emotional support animal?

☐ Service animal
☐ Emotional support animal
☐ Neither

Describe the type and number of animal(s):

Medical Clearance

Is Member medically stable for discharge?	☒ Yes	☐ No
Has Member been cleared for communicable diseases?	☒ Yes	☐ No
Is Member psychiatrically stable for discharge?	☒ Yes	☐ No
Is Member independent with ADLs?	☒ Yes	☐ No

Note: If 'No' box is checked, STOP. Member does not meet eligibility criteria.

Additional Needs: *(check all that apply)*

☐ Requires Anticoagulants
☐ Requires Injectable Medications
☐ Requires INR/PT/PTT checks
☐ Other:

Please attach the documents listed below, if available. Be sure to provide supporting documentation for areas listed above.

1. **UM Referral Form**
2. **Facesheet**
3. **History and Physical**
4. **Medication List**
5. **Wound Care Notes, if applicable**
6. **Psych Notes, if applicable**
7. **Labs/Tests/Vital Signs**
8. **Chart Notes**
9. **Supporting documents such as Social Worker Notes, Nurse Progress Notes, Paramedic Notes, etc.**

This Request Does Not Guarantee Eligibility. Check Eligibility Prior to Rending Service.
Payment Will Not Be Made For Unauthorized Services.

[redacted] [redacted] PAGE 1

ORDER RECORD

[redacted] [redacted]

OTHER PROV

[redacted]

CATEGORY CMN PROCEDURE CASEMGSDOH - Consult Case Management-S[redacted] STATUS TRA
PRIORITY ROUTINE QUANTITY SOURCE P [redacted]

SIGNED BY

AUDIT TRAIL
1 [redacted] Order ENTER in OM
2 [redacted]
3 Order Source: PROTOCOL q
4 order acknowledged
5 order viewed from Order Management
6 order viewed from Order Management
7 order viewed from Order Management
8 order viewed from Order Management
9 order viewed from Order Management
10 [redacted] order viewed from Order Management

.

Patient has instability or unmet needs related to: Living situation
Transportation
Food

[redacted]
Hospitalist History & Physical
[redacted] REPORT STATUS: Signed
[redacted] TIME: 1830

[redacted] AGE: 59 SEX: F [redacted]
[redacted] REP SRV TM: 1830
* ALL edits or amendments must be made on the electronic/computer document *

History of Present Illness

HPI
HPI:
59-year-old female with a past medical history of asthma, hypertension, hyperlipidemia, and seizures presented for shortness of breath, productive cough, and sore throat for the past couple of days. Patient with a history of asthma and recurrent pneumonia and asthma exacerbation, and last pneumonia was i[redacted] The patient is not a smoker; however, a lot of her family smoke multiple times a day for multiple years. She is on 2 L oxygen at home as needed. From the above history, it is a concern for undiagnosed COPD with COPD exacerbation. The patient is not following a Pulmonologist. The patient complains of shortness of breath, productive coughing with green and sore throat. The patient denies any blurry vision or difficulty speaking.

PMH: As above
Social history: Denies smoking tobacco, alcohol use, recreational drug use; Extensive 2nd hand smoking.

In the ED, O2 was 88 on room air. 95% on 3 L NC. The rest of the vitals are unremarkable. Labs were significant for mild thrombocytopenia with 147 K. mild hypokalemia with [redacted] Elevated CO2 to 37. LFTs were elevated to 117 AST and 76 ALT. Trop [redacted] > [redacted] Chest x-ray was unremarkable.

History
Smoking status for patients 13 years old or older: Never Smoker

Medication/Allergy-Vaccine Hx
Allergies:
Coded Allergies:
metronidazole (Severe, LOSS OF CO[redacted]
Penicillins (UNK[redacted]
acetaminophen (From NORCO) (UNK[redacted]
apple (UNK[redacted]
carrot (THROAT CLOS[redacted]
clarithromycin (LOSS OF CONSCIOUSNESS[redacted]

[redacted]

cyclobenzaprine [redacted]
hydrocodone (From NORCO) (UNKN[redacted]
oxycodone (From PERCOCET) (ITCHNESS[redacted]

Review of Systems

Free Text ROS Notes

Free Text ROS Notes:

Constitutional: No weight loss, fever, chills, weakness or fatigue.
HEENT: No change in vision or hearing. Denies sneezing, congestion, runny nose or sore throat.
Skin: No rash or itching.
Cardiovascular: Positive chest pain. No chest pressure or chest discomfort. No palpitations or pedal edema.
Respiratory: Positive shortness of breath, cough and green sputum production.
Gastrointestinal: No anorexia, nausea, vomiting or diarrhea. No abdominal pain or blood in stool.
Genitourinary: No burning micturition. No urinary frequency or incontinence.
Neurologic: No headache, dizziness, syncope, unilateral weakness, ataxia, numbness or tingling in the extremities. No change in bowel or bladder control.
Musculoskeletal: No muscle pain, back pain, joint pain or stiffness.
Hematologic: No bleeding or bruising.
Lymphatics: No enlarged lymph nodes.
Psychiatric: No depression or anxiety.
Endocrine: No reports of sweating. No cold or heat intolerance. No polyuria or polydipsia.

Objective

General

VS/I&O:

Vital Signs:

Date Time	Temp	Pulse	Resp	B/P	B/P Mean	Pulse Ox	O2 Delivery	O2 Flow Rate	FiO2
[redacted]	[redacted]	96	15	118/79	[redacted]	96	Nasal cannula		
	[redacted]	93	18	138/82	100.5	95	Nasal cannula	3	
		99	21	117/65		96	Nasal cannula	2	
	[redacted]	93	19	108/60		95	Nasal	2	

[redacted]

[redacted]							cannula		
							Nasal cannula	2	
							Nasal cannula	3	
		85	15	106/63		96	Nasal cannula	2	
							Nasal cannula	2	
		83	14	113/77		95	Nasal cannula	2	
	[redacted]	94	12	125/78		96	Nasal cannula	4	

24 hour I&O ending at 0700:

	[redacted]	
Intake Total		
Output Total		
Balance		
Patient Weight		72.727 kg
Weight Measurement Method		Stated/Reported

PATIENT WEIGHT:

Weight (lb):
Weight (oz):
Weight (kg): 72.727

Results

Findings/Data:

Laboratory Tests

	[redacted]			
	2252	1855	1855	1618
Chemistry				
Hemoglobin A1c [redacted] - [redacted] %)			6.9 H	
Lactic Acid [redacted] - 2 [redacted] mmol/L)		[redacted]		
Troponin I High Sens (2.500 - 45.000 ng/L)	58.960 H			53.880 H

[redacted]

	[redacted]		
	1618	1413	1411
Chemistry			
Sodium (136 - 145 mmol/L)			138
Potassium [redacted] mmol/L)			3.4 L
Chloride (98 - 107 mmol/L)			95 L
Carbon Dioxide (20 - 31 mmol/L)			37 H
Anion Gap (5 - 15 mmol/L)			6
BUN (9 - 23 mg/dL)			11
Creatinine [redacted] - 1 [redacted] mg/dL)			[redacted] L
Estimated GFR			108
BUN/Creatinine Ratio [redacted] - [redacted] RATIO)			21.8 H
Glucose (74 - 106 mg/dL)			107 H
Calculated Osmolality (275 - 295 MOSM/KG)			276
Calcium [redacted] - [redacted] mg/dL)			[redacted]
Phosphorus [redacted] - [redacted] mg/dL)	5.0 H		
Magnesium [redacted] - [redacted] mg/dL)	[redacted]		
Total Bilirubin [redacted] - [redacted] mg/dL)			[redacted]
AST (13 - 40 U/L)			117 H
ALT (7 - 40 U/L)			76 H
Total Alk Phosphatase (46 - 116 U/L)			76
Troponin I High Sens (2.500 - 45.000 ng/L)			55.740 H
B-Natriuretic Peptide (0 - 100.0 pg/mL)		9	
Serum Total Protein [redacted] - [redacted] g/dL)			[redacted]
Albumin [redacted] - [redacted] g/dL)			[redacted]
Globulin [redacted] - [redacted] g/dL)			3.8
Albumin/Globulin Ratio [redacted] - [redacted] RATIO)			0.9 L
TSH [redacted] - [redacted] uIU/mL)	2.589		
Spec Hemolysis Index			1

Laboratory Tests

	[redacted]
	1413
Hematology	
WBC [redacted] - [redacted] K/mm3)	[redacted]
RBC (4.2 - [redacted] M/mm3)	[redacted] L
Hgb [redacted] - [redacted] gm/dL)	[redacted]
Hct (35 - 48 %)	[redacted]
MCV (80 - 99 fL)	98.8
MCH (27 - 34 pg)	[redacted]
MCHC [redacted] - [redacted] g/dL)	[redacted]
RDW [redacted] - [redacted] %)	[redacted]

[redacted]

Plt Count (150 - 400 K/mm3)	148 L
MPV [redacted] - [redacted] FL)	9.3 L

Laboratory Tests

	[redacted]
Serology	
Influenza Type A Ag (NEGATIVE)	NEGATIVE
Influenza Type B Ag (NEGATIVE)	NEGATIVE
SARS Antigen (LFIA) (NEGATIVE)	NEGATIVE

Laboratory Tests

	[redacted]
Toxicology	
Plasma/Serum Alcohol [redacted] mg/dL)	[redacted]

Radiology data:
Recent Impressions:
RADIOLOGY - CHEST 1 VIEW [redacted]
*** Report Impression - Status: SIGNED Ent [redacted]

FINDINGS/IMPRESSION:

1. No pneumothorax.
2. Normal lung volumes. The lungs are grossly clear. No consolidation.
3. Cardiomediastinal silhouette is within normal limits. No pulmonary vascular congestion.No pleural effusion.
4. No significant osseous abnormality.

Impression By [redacted]

Free Text Obj Notes

Free Text Obj Notes:
General: Alert and oriented
Mental Status: Normal affect.
Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Ears and nose without

[redacted]

masses, lesions or deformities.
Neck: Supple, Full range of motion.
Respiratory: Diffuse moderate expiratory wheezing in all lung fields. no rales or rhonchi.
Cardiovascular: RRR, no murmurs, rubs or gallops. S1-S2 normal
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds. No pulsatile mass. No hepatosplenomegaly.
Genitourinary: No costovertebral angle tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits. Sensation intact bilaterally.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Diagnosis, Assessment & Plan

Free Text DxA&P Notes

Free Text DxA&P Notes:
59-year-old female with a past medical history of asthma, hypertension, hyperlipidemia, and seizures presented for shortness of breath, productive cough, and sore throat for the past couple of days.

acute hypoxic respiratory failure (AHRF) secondary to below
asthma exacerbation versus COPD exacerbation
NSTEMI; likely type 2 secondary to AHRF
hypercapnia
>[redacted]>[redacted]>[redacted]

- start IV Rocephin 1g Q24H[redacted]
- start IV doxycycline 100 mg b.i.d[redacted]
- Patient had a hx of reaction of LOC to Calirthromycin; to avoid cross activity, will avoid azithromycin and start Doxcycline.
- start scheduled DuoNeb q.4 hours
- start Solu-Medrol 100 mg q.6 hours.
- sputum culture - pending results
- wean off oxygen as tolerated
- IVF LR maintenance
- continue to monitor

- trend troponin to peak
- cardiology consulted - pending recs

Hypertension

[redacted]

- continue home amlodipine 5 mg q.d.

transaminitis
hyperlipidemia
> AST: 117, ALT: 76

- hold home home atorvastatin 40 mg q.d. in the setting of transaminitis
- order alcohol level
- continue to monitor

History of seizures
- Hold home valproic acid 250 mg q.8 hours in the setting of transaminitis.
- start Keppra 500 mg b.i.d. in place of home valproic acid 250 mg Q 8 hours
- continue home gabapentin 600 mg t.i.d.
- continue home duloxetine 60 mg q.d.

F: LR maintenance fluids
E: Replete p.r.n.
N: Cardiac healthy diet
G: N.
V: Lovenox 40 mg

Code status: Full code

Dispo: Cardio recs, culture results, LFT imrpoves, clinical improvement

The patient discussed and seen with attending, Dr[redacted]rees with plan. All questions were answered at bedside. Total time spent was more than 44 minutes.

MIPS/Quality Measures
MIPS Measures MIPS Measures

Quality Measures
Quality Measures Quality Measures

Electronical[redacted]
Electronical[redacted]

[redacted]
END OF REPORT

[redacted]

Medication Orders History w/ Administrations

[redacted]

Age/Sex: 59 F

[redacted]

Med Admin days: 2, DC'd days: 2

Name	Dose	Sig/Sch	Route	Not Given Date/Time	Start	Stop	Note
ACETAMINOPHEN	650 MG	Q6H PRN/	PO				
CEFDINIR	300 MG	BID	PO				
	300 MG						
	300 MG						
	300 MG						
	300 MG						
DOXYCYCLINE HYCLATE	100 MG	BID	PO				
	100 MG						
	100 MG						
	100 MG						
	100 MG						
DOXYCYCLINE HYCLATE	100 MG	Q12H	IV				
[redacted] SODIUM CHLORID	100 ML						
DULoxetine HCL	60 MG	DAILY	PO				
	60 MG						
	60 MG						
ENOXAPARIN SODIUM	40 MG	DAILY@21	SUBQ				
	40 MG						
	40 MG						
GABAPENTIN	600 MG	TID	PO				
	600 MG						
	600 MG						
	600 MG						
	600 MG						
	600 MG						
	600 MG						
IPRATROPIUM/ALBUTEROL S	3 ML	RTQ4H	NEB				
	3 ML						
	3 ML						
	3 ML						
	3 ML						
	3 ML						
		Not Give					
	3 ML						
	3 ML						
	3 ML						
	3 ML						
	3 ML						
	3 ML						
IPRATROPIUM/ALBUTEROL S	3 ML	RTQ4H PR	NEB				
LACTULOSE	40 GM	Q6H PRN/	PO				
MAGNESIUM SULF 2000MG/5	50 ML	DAILY PR	IV				
METHYLPREDNISOLONE SOD	40 MG	Q6H	IV				
	40 MG						
	40 MG						
	40 MG						

[redacted]

[redacted]

Medication Orders History w/ Administrations

[redacted]

Age/Sex: 59 F [redacted]

Med Admin days: 2, DC'd days: 2

Name	Dose	Sig/Sch	Route	Date/Time	Start	Stop	Note
				Not Given			
METHYLPREDNISOLONE SOD	100 MG	Q6H	IV				
MUPIROCIN	1 APPLIC	BID	NASAL				
	1 APPLIC						
	1 APPLIC						
	1 APPLIC						
	1 APPLIC						
PANTOPRAZOLE SOD SESQUI	40 MG	DAILY	PO				
	40 MG						
	40 MG						
POTASSIUM CHLORIDE	100 ML	ASDIR/PR	IV				
POTASSIUM CHLORIDE	40 MEQ	DAILYP/P	PO				
RINGERS SOLUTION,LACTAT	1,000 ML	ASDIR	IV				
SENNA/DOCUSATE SODIUM	2 TAB	BID PRN/	PO				
SIMETHICONE	80 MG	Q4H PRN/	PO				
SOD BIPHOS/POT PHOSPHAT	2 PACKET	DAILYP/P	PO				
SODIUM CL FOR INHALATIO	3 ML	RTQ4H	NEB				
		Not Give					
		Not Give					
		Not Give					
		Not Give					
		Not Give					
		Not Give					
		Not Give					
		Not Give					
VALPROIC ACID	250 MG	Q8H	PO				
	250 MG	Early: p					
	250 MG						
	250 MG						
	250 MG						
	250 MG						
	250 MG						
amLODIPine	[redacted] MG	DAILY	PO				
	[redacted] MG						
	[redacted] MG						
bisacodyL	10 MG	DAILYP/P	RECTA				
cefTRIAXone SODIUM	1,000 MG	Q24H	IV				
IN:0.9% SODIUM CHLORID	100 ML						
diphenhydrAMINE HCL	25 MG	Q4H PRN/	PO				
guaiFENesin	400 MG	Q4H PRN/	PO				
	400 MG						
	400 MG						
	400 MG						
hydrOXYzine HCL	25 MG	Q6H PRN/	PO				
levETIRAcetam	500 MG	BID	PO				
		Not Give					
		Not Give					
		Not Give					
		Not Give					

[redacted]

[redacted]

Medication Orders History w/ Administrations

[redacted]

Age/Sex: 59 F [redacted]

Med Admin days: 2, DC'd days: 2

Name	Dose	Sig/Sch	Route	Not Given Date/Time	Start	Stop	Note
levETIRAcetam	500 MG	BID	PO		[redacted]		
ondansetron HCL	4 MG	Q4H PRN/	PO				
ondansetron HCL	4 MG	Q4H PRN/	IV				
predniSONE	40 MG	DAILY	PO				
	40 MG	Late: pa		[redacted]			
	40 MG						

[redacted]

View Note

Author [redacted]

Note Category: MULTIDISCIPLINARY NOTES

Date Time by

Occurred [redacted]

Recorded

NN: ASSUMED CARE FOR THIS PATIENT. PT IS A&OX4, VITAL SIGNS WNL. PT ON 2L NC SATING 98%. PT IS RESTING IN BED AND DENIES ANY DISTRESS OR PAIN.

Note Type	Description
No Type	NONE